

The core pathway, *explained.*

Individual assessment and triage · the psychoeducation workshop · the four-week group. What each component is, why it is built that way, and how it runs.

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01 · ENTRY AND MATCHING

Individual assessment and *triage.*

WHAT.

A two-layer entry process. Every referred patient completes digital consent, the gut-brain intake form and the EF™ triage self-assessment online — sixteen items, 0–4, across gut-focused anxiety, emotion sensitivity, health anxiety and functional impact (total /64) — before any appointment. Patients in the red band (49–64), or with risk flags or complexity, then attend a 50-minute individual assessment with Dr Anticich: presentation history, medical context, psychological formulation, and the EF™ Triage Tool (0–96) as the baseline outcome measure.

WHY.

Matching intensity to need is what makes stepped care safe and efficient. Score-assisted triage keeps entry fast and consistent; clinician confirmation keeps it clinically sound — automated scoring never allocates care on its own. The individual assessment protects the group environment (risk and complexity are identified before allocation) and gives every complex patient a formulation, not just a score. Baseline measurement is what makes outcomes reportable to MacMurray.

HOW.

Referral or self-referral > acknowledgement same day > forms and self-assessment completed online > triage decision within two business days.

Green (0–22) > education and self-guided tools · Amber (23–35) > workshop and four-week group · Orange (36–48) > group plus full curriculum · Red (49–64), risk or complexity > individual assessment first. Red flags override every band.

With consent, the referrer receives the assessment summary, EF™ profile and recommended pathway. Measures repeat at mid-point, discharge and three months.

CLINICAL GOVERNANCE

Dr Sarah Anticich is clinician of record for every triage decision and risk escalation. The self-assessment is a structured clinical aid, not a validated psychometric instrument — bands inform, the clinician decides.

The psychoeducation *workshop*.

WHAT.

A half-day introductory DGBI workshop (full-day option available) delivered in a group psychoeducation format. Content: the gut–brain axis as bidirectional physiology; what sustained symptom stress does to regulation, thinking and behaviour (The Tunnel™); the C³ Loop™ — Centre › Clarify › Choose; and first skills, including the physiological sigh and the S.H.F.T.™ protocol (Stop · Hold · Focus · Take Action). Participants leave with the starter toolkit and a clear map of the pathway.

WHY.

Education is the least intensive intervention that works — for green–amber presentations, accurate physiology plus first regulation skills is often enough to break the reassurance cycle. It is also the lowest-threshold entry point: no confirmed diagnosis required, no commitment to therapy implied, and no suggestion that symptoms are imagined. For hesitant patients, a workshop is an education event, not a psychology appointment — which is exactly why they attend.

HOW.

Open to referred patients and self-referrals; suited to green–amber bands and to any patient you want engaged before a specialist review.

Group psychoeducation format with interactive exercises; personal disclosure is not required. Workbook and tools included.

Booked at triage or directly at enquiry; dates confirmed at booking, payment at booking, Southern Cross affiliated. After the workshop: step down to self-guided tools and eLearning, or step up to the four-week group.

The four-week *group*.

WHAT.

The Foundations group — four weekly sessions in a small closed group, workbook included, structured on the C³ spine. The arc: regulate first (nervous-system skills, S.H.F.T.[™], soothing systems); then restore accurate thinking (fact vs story, hypervigilance and catastrophic-interpretation tools); then act (graded re-engagement with food, activity and life); closing with integration and a personal maintenance plan. Between sessions, patients run zone check-ins and tracking logs.

WHY.

Group is deliberately the primary treatment pathway, not the budget option. Normalisation and reduced shame do clinical work that individual sessions cannot — patients discover their symptoms are a recognised pattern, not a personal failing — and co-regulation and shared modelling accelerate skills uptake. It is also the scalable format: one clinician, a whole cohort, measured outcomes.

HOW.

Entry via triage (amber–orange band) or step-up from the workshop. The referral risk screen keeps group-unsuitable presentations out; they are seen individually first.

Weekly sessions with a consistent group; confidentiality and group agreements set in session one; participation is always at the patient's own pace.

Pre and post measures (IBS-SSS, DASS-21, VSI, EF[™] Triage Tool); discharge summary to the referrer with consent; step-up to individual work or step-down to self-paced eLearning as indicated.

CONTACT

Refer or discuss a case — consultant@sarahanticich.co.nz · executiveframe.co.nz/health-dgbi.html · triage within two business days · 15-minute case discussions welcome.

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